

Response to HHS Request for Information: Artificial Intelligence in Healthcare

A Salutogenic Perspective

Submitted by:

Shimon Waldfogel MD

Institute for Salutogenesis

Shimonwaldfogel@gmail.com

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WHO IS RESPONDING

I am a physician with 30 years of clinical practice in psychiatry and medical education, a current AI developer and experimenter building tools to facilitate health creation and maintenance, and President of the Institute for Salutogenesis — a non-profit organization with a mission to empower individuals and communities to thrive by focusing on the origins of health rather than just disease.

THE PERSPECTIVE I BRING

The Salutogenic Reframe

The RFI focuses on how AI can make current healthcare more efficient: faster diagnosis, optimized scheduling, reduced administrative burden, streamlined billing. These applications matter. But they accept a fundamental assumption: that healthcare's primary purpose is managing disease.

I propose a different starting point.

Salutogenesis — the study of what creates and maintains health — asks a fundamentally different question than pathogenesis, which studies what causes disease. This is not semantic. It changes everything about what we ask AI to accomplish.

The current approach asks: How can AI help us diagnose and treat disease more efficiently?

The salutogenic approach asks: How can AI help everyone get what they need to stay healthy and flourish?

One optimizes disease processing. The other creates conditions for health. Both are necessary. Current policy supports only the first.

Why This Matters Now

America spends nearly twice as much per capita on healthcare as peer nations, yet has shorter lifespans, higher rates of chronic disease, a growing mental health crisis, and widening health disparities.

This is not an efficiency problem. We are very efficient at managing disease.

This is a priority problem. We lack systematic support for health creation. AI offers unprecedented opportunity to address both — enhance disease management and create conditions for health — but only if policy enables it.

WHAT SALUTOGENIC AI REQUIRES

To realize AI's potential for health creation, four foundational elements must be addressed:

1. Shared Decision-Making as Core Design Principle

Current Reality:

Most AI in healthcare serves providers and administrators. Patients are data sources and intervention targets, not participants.

Salutogenic Requirement:

AI tools must support genuine shared decision-making — not automated recommendations patients passively accept, but systems that help people understand their options, articulate their values, and make informed choices aligned with their lives. This means:

- Transparent explanation of how AI reaches recommendations
- Clear, patient-appropriate presentation of alternatives and tradeoffs
- Support for patients to question and challenge clinician and AI guidance
- Integration of patient preferences and values into decision-making
- Tools that strengthen rather than replace the patient-provider relationship

Policy Implication: Federal funding and reimbursement should require AI systems to demonstrate meaningful patient engagement, not just clinical accuracy.

2. Access to Integrated Data Across Silos

Current Reality:

Health is determined by clinical factors, social determinants, environmental exposures, community resources, and behavioral health — all tracked in separate, incompatible systems.

Salutogenic Requirement:

AI cannot support whole-person health creation with fragmented data. We need privacy-protected integration across:

- Clinical records (EHR systems)
- Social services (housing, food assistance, employment support)
- Community resources (nonprofits, local programs, support networks)

- Behavioral health (mental health and substance use treatment)
- Environmental data (housing quality, neighborhood safety, pollution exposure)

The Barrier:

As an AI developer actively building these tools, I cannot create effective health creation systems without this integration. Current data silos make whole-person AI technically impossible, not just difficult.

Policy Implication: HHS should establish frameworks for privacy-protected data integration across health and social services. Without this, AI will remain limited to narrow clinical applications.

3. Clinician Competence in Using AI Tools

Current Reality:

AI is being deployed in clinical settings where providers have minimal training in whole-person health approaches, AI capabilities and limitations, appropriate interpretation of AI recommendations, and patient communication about AI-influenced decisions.

Salutogenic Requirement:

Health creation AI will be used by diverse professionals — physicians, nurses, community health workers, care navigators, social workers — who need competence in:

- Using AI to support rather than replace clinical judgment
- Identifying when AI recommendations enhance versus undermine shared decision-making
- Explaining AI guidance in patient-comprehensible terms
- Coordinating AI-enabled interventions across disciplines

Policy Implication: HHS should fund development of AI competency curricula for all health professions and require AI literacy as part of continuing education.

4. Focus on Social Determinants and Community Resources

Current Reality:

Multiple commenters on this RFI have noted the importance of addressing social determinants of health (SDOH). I strongly agree — and want to emphasize what this actually requires. Screening for food insecurity, housing instability, transportation barriers, and social isolation is valuable only if it connects people to resources. Most current SDOH screening ends with documentation in the medical record. The patient still faces the same barriers. Nothing changes.

Salutogenic Requirement:

AI tools must:

- Screen for social determinants systematically
- Automatically match people with available community resources
- Generate referrals to social services and community organizations
- Track whether connections were successful

- Identify systematic gaps where resources don't exist
- Support community health workers and care navigators with real-time information

Policy Implication: HHS should establish national standards for community resource data and fund infrastructure enabling clinical-community integration.

RESPONDING TO QUESTION 6: GREATEST POTENTIAL AI TOOLS

Given this salutogenic perspective, what AI tools have the greatest potential to improve outcomes and reduce costs?

The Answer: Integrated Whole-Person Support Systems

AI tools with greatest potential will address the whole person — integrating across dimensions current systems treat separately:

1. Support Health Creation Alongside Disease Management

Not replacing current clinical AI, but complementing it with systems that help people understand what creates health in their specific circumstances, connect them to resources that promote flourishing, support prevention before disease develops, and make healthcare comprehensible and navigable.

2. Address the Whole Person

Integrating across dimensions current systems treat separately:

- Physical health (clinical care, prevention, health behaviors)
- Mental and emotional wellbeing (stress, resilience, psychological health)
- Social determinants (housing, food, economic security, community connection)
- Environmental factors (neighborhood conditions, exposures, safety)
- Spiritual factors (promoting evidence-based practices and community connection where desired)

3. Enable Shared Decision-Making

Tools that translate complexity into comprehensible information, present options and tradeoffs clearly, support informed choice aligned with patient values, and strengthen rather than supplant the provider-patient relationship.

4. Connect Clinical Care with Community Resources

Systems that identify social barriers to health, match people with available community resources in real time, generate and track referrals, and close feedback loops that identify gaps requiring community investment.

5. Focus on Critical Developmental Windows

Especially the first thousand days (pregnancy through age 2), when:

- Health trajectories are established
- Interventions have compounding lifetime effects
- Family support prevents intergenerational health disparities
- Return on investment is highest (\$4–\$9 return per dollar invested)

WHY THESE TOOLS HAVE GREATEST POTENTIAL

Making America Healthy requires creating health — not just managing disease more efficiently. These tools operationalize that vision. The cost reductions they generate are not the goal; they are the consequence of doing something more fundamental: building the conditions under which people actually thrive.

Salutogenic Outcomes: What Health Creation Looks Like

The metrics that matter for health creation are not simply the absence of pathology. They are the presence of the conditions that allow people to flourish. Drawing on Aaron Antonovsky's foundational framework, these outcomes include:

Sense of Coherence

People's capacity to understand their situation, believe they have the resources to manage it, and find meaning in doing so. Research consistently shows this is one of the strongest predictors of sustained health across the lifespan. AI tools designed around health creation should be measured in part by whether they strengthen this capacity — not just whether they route people to clinical services.

Agency and Navigational Capacity

Not simply connecting people to resources, but building their capacity to navigate complex systems on their own terms over time. This is a fundamentally different outcome metric than reduced utilization. A person who understands how to advocate for themselves, interpret clinical information, and access community support is not just avoiding illness — they are accumulating health.

Community Health Assets

These tools identify and strengthen what already exists in communities — treating local assets as generative resources rather than their absence as the primary problem. The goal is a map of abundance, continuously updated, that connects people with the social infrastructure of health that surrounds them.

Developmental Trajectories

Particularly through the First 1000 Days lens, salutogenic outcomes compound. A child born into a family with strong coherence, social support, and navigational capacity doesn't merely avoid adverse outcomes — they develop differently. The neurological, relational, and emotional foundations laid in those first years shape a lifetime of health. This is the highest-return investment in health creation available.

System Intelligence: Generating New Knowledge

These tools do not merely deliver existing knowledge more efficiently. They generate new knowledge — about how social determinants, community assets, and clinical care interact in specific populations; about which interventions actually build coherence and capacity rather than just managing symptoms; about where systematic gaps in community resources require investment.

This adaptive capacity — a learning system that improves its understanding of what creates health — represents a qualitatively different contribution than optimization of existing workflows.

Disease Burden Reduction as Consequence

When health creation succeeds, reductions in disease burden follow. Emergency department visits for conditions manageable in community settings decline — not because we redirected people, but because those conditions didn't develop. Hospital readmissions driven by social barriers fall — not because we managed transitions better, but because the underlying barriers were addressed. Chronic conditions are prevented or substantially delayed — not through screening and surveillance, but through the early, sustained support that allows people to maintain health.

These are not marginal efficiency gains. They represent a fundamental reduction in disease burden — durable, compounding, and equitable in a way that disease management optimization can never be.

CRITICAL BARRIERS FROM DEVELOPMENT EXPERIENCE

As someone actively building these tools, I face specific barriers that have clear policy solutions:

Data Silos: Cannot integrate across clinical, social service, community resource, and behavioral health systems.

Regulatory Uncertainty: Unclear whether health creation tools are medical devices, wellness products, or something requiring new classification.

No Reimbursement Pathway: Current payment models reimburse disease management, not health creation. No viable path to sustainability for tools that prevent the conditions reimbursement depends on.

Lack of Standards: No national standards for community resource data, clinical-community referral protocols, or outcome measurement for social interventions.

Each barrier has a policy solution. What is required is recognition that health creation AI faces different obstacles than disease management AI — and deserves deliberate support.

RECOMMENDATIONS

Immediate

- Issue guidance clarifying regulatory pathways for salutogenic AI tools

- Convene stakeholders on data integration barriers across HHS and related Department and Agencies (HHS Genesis Mission)
- Make available easy-to-use APIs for available federal health data
- Support AI in Health Hackathons with HHS staff actively accessible

Short-Term

- Develop standards for community resource data and clinical-community integration
- Create payment codes for whole-person assessment and SDOH intervention
- Fund development and implementation of AI competency training for all health professions
- Prioritize First 1000 Days applications in research funding, including through SBIR

Medium-Term

- Establish a formal “Health Creation AI” regulatory designation
- Build national community resource data infrastructure
- Implement value-based payment models that reward prevention and health creation
- Expand Medicaid coverage for AI-enabled preventive and salutogenic services

CONCLUSION

AI in healthcare stands at a choice point.

One path accelerates adoption of tools that make current disease management more efficient. The other enables tools that create health alongside managing disease. The first path is well-supported by current policy. The second requires deliberate intervention.

As clinician, experimenter, developer, and salutogenesis advocate, I am actively building tools for the second path. I know they can work technically, and I know they can contribute meaningfully to the health and wellbeing of Americans.

The perspective I bring — grounded in clinical reality, informed by development experience, and guided by salutogenic principles — suggests that AI's greatest potential lies not in processing disease-related information faster, but in creating health more effectively.

This aligns with HHS's mission, serves MAHA's vision, and addresses what patients actually need.

The question is whether policy will enable it.